

American Association of Physicists in Medicine Response: Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care

<https://www.federalregister.gov/documents/2025/12/23/2025-23641/request-for-information-accelerating-the-adoption-and-use-of-artificial-intelligence-as-part-of>

Executive Summary

The American Association of Physicists in Medicine (AAPM) appreciates the opportunity to provide comments to the Department of Health and Human Services (HHS) on accelerating the adoption and use of artificial intelligence (AI) as part of clinical care. AAPM is a professional society representing more than 9,000 medical physicists and scientists dedicated to advancing the safe and effective use of physics and engineering in medicine. The following comments reflect the collective expertise of AAPM medical physicist members who operate at the intersection of advanced technology, clinical practice, regulatory oversight, and patient safety, and offer a perspective grounded in both innovation and real-world implementation within highly regulated healthcare environments.

AAPM's recommendations emphasize that successful AI adoption in safety-critical clinical domains depends not only on technological advancement, but also on validation, lifecycle monitoring, governance clarity, interoperability, workforce preparedness, and aligned incentives. In healthcare environments where AI tools may influence diagnosis, treatment planning, and patient management, innovation must be accompanied by structured oversight, evidence generation, and transparent accountability. Federal policies should recognize these realities and support coordinated ecosystems that integrate technology developers, healthcare institutions, regulators, payers, and professional societies.

Across the RFI topics, AAPM highlights several cross-cutting priorities:

- **Strengthening validation and benchmarking infrastructure** by investing in shared reference datasets, independent testing frameworks, post-deployment monitoring systems, and clinically meaningful performance metrics that enable objective evaluation and sustained oversight of AI tools used in patient care.
- **Establishing risk-based governance and oversight models** that align regulatory expectations with clinical impact, clarify lifecycle accountability, and provide consistent guidance for non-medical-device AI embedded in healthcare workflows.
- **Aligning reimbursement and programmatic incentives with measurable clinical value** by modernizing CMS payment and quality programs to reward demonstrable improvements in safety, efficiency, consistency, and patient outcomes enabled by validated AI applications.

- **Advancing interoperability and data harmonization** through standardized data models, integration of imaging and structured clinical data, and inclusion of audit logs, version control, and performance metadata necessary for scalable validation and benchmarking.
- **Developing workforce capacity and defined oversight roles** by supporting role-based competency frameworks and training pathways for professionals responsible for AI commissioning, quality assurance, monitoring, and governance.
- **Supporting private-sector standards and profession-led assurance activities** that complement federal oversight, reduce duplicative requirements, and ground AI governance expectations in established clinical safety practices.
- **Expanding implementation science and economic evaluation research** to generate evidence on cost-effectiveness, workflow impact, and long-term clinical outcomes associated with AI-enabled care.
- **Promoting transparency, patient trust, and equitable access** by encouraging clear communication regarding AI use, supporting bias mitigation and fairness research, and ensuring validated technologies can be responsibly adopted across diverse healthcare settings.

AAPM looks forward to continued engagement with HHS and its agencies to develop policies that accelerate responsible AI innovation while strengthening the validation and governance frameworks necessary to protect patients and sustain public trust.

(i) What are the biggest barriers to private sector innovation in AI for health care and its adoption and use in clinical care?

AI innovation in healthcare is increasingly constrained not by algorithm development capacity, but by gaps in validation standards, governance clarity, implementation infrastructure, and workforce readiness. In safety-critical domains such as medical imaging and radiation therapy, AI tools are being introduced into complex, regulated clinical environments that require rigorous commissioning, quality assurance, and lifecycle oversight. However, many AI systems, particularly those not regulated as medical devices, lack consistent frameworks for evaluation, monitoring, and accountability. These structural uncertainties slow adoption, increase institutional risk, and limit the scalability of otherwise promising technologies.

AAPM identifies the following primary barriers and corresponding federal opportunities:

- **Lack of standardized validation and benchmarking frameworks.**
AI tools are frequently evaluated using heterogeneous datasets and metrics, making it difficult for healthcare institutions to assess generalizability across patient populations, equipment platforms, and care settings. Federal support for nationally recognized validation standards, shared benchmarking datasets, and transparent performance reporting would reduce uncertainty and strengthen institutional confidence.
- **Regulatory and lifecycle ambiguity.**
AI systems may be continuously updated or adapted after deployment, yet guidance regarding post-market monitoring, retraining, performance drift, and institutional governance responsibilities remains inconsistent. Clear federal expectations regarding lifecycle oversight, documentation, and accountability would reduce risk and improve implementation consistency.
- **Fragmented data infrastructure and limited interoperability.**
High-quality AI development and validation depend on diverse, well-curated datasets, but healthcare data remain siloed and inconsistently structured. Investment in interoperable data standards, federated data-sharing models, and privacy-preserving collaboration mechanisms would improve dataset representativeness and accelerate safe innovation.

- **Insufficient workforce training and institutional governance capacity.**

Effective AI integration requires professionals capable of critically evaluating model performance, identifying bias, overseeing commissioning, and managing lifecycle monitoring. Federal workforce development initiatives that incorporate AI literacy, validation principles, and governance frameworks into clinical education would support responsible adoption.

By addressing validation gaps, clarifying governance expectations, strengthening data infrastructure, and investing in workforce capacity, HHS can reduce implementation risk and enable safe, scalable, and evidence-based AI adoption across clinical settings.

(ii) What regulatory, payment policy, or programmatic design changes should HHS prioritize to incentivize the effective use of AI in clinical care and why? What HHS regulations, policies, or programs could be revisited to augment your ability to develop or use AI in clinical care? Please provide specific changes and applicable Code of Federal Regulations citations

AI adoption in clinical care will accelerate only if regulatory expectations are clear, governance responsibilities are defined, and reimbursement structures align with validated improvements in safety and quality. In safety-critical domains such as medical imaging and radiation therapy, institutions must integrate AI into existing quality assurance programs, documentation systems, and clinical workflows. Policy uncertainty or misaligned incentives can delay adoption even when tools demonstrate measurable clinical value.

AAPM recommends the following priority policy actions:

- **Establish risk-based oversight guidance for AI used in clinical care that aligns expectations with clinical impact rather than device classification alone.**

HHS should publish cross-agency guidance clarifying governance expectations for AI tools that influence clinical decision-making, including commissioning, documentation, performance monitoring, and update management. Oversight intensity should scale with clinical risk and potential patient impact, reducing unnecessary burden for low-risk tools while preserving safeguards for high-impact applications.

- **Clarify how existing HHS authorities apply to AI model development, deployment, and monitoring, particularly under HIPAA privacy and security requirements.**

Healthcare institutions require clear guidance on permissible data uses for AI development, validation, and operational monitoring. HHS should issue interpretive guidance clarifying how healthcare operations, quality assessment, and performance improvement activities involving AI are treated under the HIPAA Privacy Rule and Security Rule (see **45 C.F.R. Part 164, Subparts C and E**). Clear expectations regarding de-identification, audit controls, and ongoing model monitoring would reduce compliance uncertainty.

- **Modernize CMS payment structures to ensure validated AI-enabled improvements in safety and quality are financially sustainable.**

AI tools that reduce errors, improve consistency of care, enhance workflow efficiency, or support earlier detection should not face unintended reimbursement barriers. CMS should review value-based payment models and hospital payment structures to ensure they are compatible with investments in validated AI tools and do not create disincentives for adoption.

- **Incorporate validated AI-enabled safety and quality improvements into CMS quality reporting frameworks where appropriate.**

CMS quality programs strongly influence institutional priorities and investment decisions. CMS

should allow AI-enabled improvements in safety, accuracy, or efficiency to be recognized within relevant quality reporting and performance measurement programs under the Quality Payment Program framework (see **42 C.F.R. Part 414, Subpart O**) and applicable hospital quality reporting regulations, when supported by objective evidence.

(iii) For AI tools that are not regulated as medical devices, what novel legal or implementation issues exist, and what role should HHS play in addressing them?

AI systems that influence clinical decision-making but are not regulated as medical devices introduce governance and accountability challenges that are not fully addressed by existing frameworks. These tools may support triage, workflow prioritization, image analysis, documentation, or decision support functions without falling squarely within traditional device classifications. As a result, institutions face uncertainty regarding responsibility for validation, oversight, monitoring, liability exposure, and lifecycle management.

Unlike traditional software tools, many AI systems are dynamic. Models may be updated by vendors, retrained on new datasets, or fine-tuned locally. Without clear guidance, healthcare organizations must independently determine how to manage version control, performance drift, documentation, and change management within existing quality systems. This variability creates inconsistent oversight practices and may increase institutional risk.

AAPM recommends the following federal actions:

- **Clarify institutional and vendor accountability expectations for non-device AI used in clinical care.**
HHS should provide guidance outlining shared responsibilities among developers, vendors, and healthcare institutions when AI tools influence clinical workflows or patient care decisions. Clear expectations regarding documentation, validation prior to deployment, and ongoing monitoring would reduce liability ambiguity.
- **Provide guidance on lifecycle management and post-deployment oversight.**
HHS should clarify expectations related to model updates, retraining, performance drift monitoring, and documentation practices. Institutions require clear standards for when revalidation is necessary and how changes should be recorded within quality assurance systems.
- **Encourage transparent contractual practices that preserve institutional oversight.**
Healthcare organizations should retain sufficient access to performance data, audit rights, and documentation to independently evaluate AI tools used in their facilities. HHS can promote model transparency principles and best practices that support institutional governance without mandating disclosure of proprietary code.
- **Develop model governance frameworks and toolkits for health systems.**
HHS could support the development of template governance structures, oversight committee models, commissioning checklists, and escalation protocols to assist institutions in safely integrating non-device AI into clinical environments.

(iv) What evaluation methods, metrics, and monitoring approaches are most promising for AI tools used in clinical care, and how should HHS support their development and implementation?

Effective AI adoption in clinical care depends on rigorous pre-deployment validation and sustained post-deployment monitoring. In safety-critical domains such as medical imaging and radiation therapy,

technologies are traditionally commissioned using structured acceptance testing, independent verification, and routine quality assurance protocols. AI systems, particularly those that are adaptive or frequently updated, require comparable lifecycle oversight to ensure continued safety and effectiveness.

Pre-deployment evaluation should rely on clinically meaningful performance metrics that align with the tool's intended use and reflect real-world clinical conditions. Technical metrics such as accuracy or area under the curve may be insufficient without contextual measures of workflow impact, error reduction, generalizability across institutions, and performance across diverse patient populations.

Post-deployment monitoring is equally important. AI models may experience performance drift over time due to changes in patient demographics, imaging equipment, clinical protocols, or data acquisition practices. Without systematic monitoring, degradation in performance may go undetected. Structured monitoring should include mechanisms to identify data drift, assess adverse outcomes, evaluate bias, and determine when revalidation is required.

AAPM recommends the following federal actions:

- **Support development of nationally recognized validation standards and benchmarking datasets.**
HHS should invest in shared reference datasets and independent benchmarking initiatives that enable objective comparison of AI tools across institutions and clinical environments.
- **Promote clinically meaningful performance metrics.**
Evaluation frameworks should emphasize patient safety, error reduction, workflow impact, and consistency of care rather than relying solely on retrospective technical performance metrics.
- **Fund research on post-deployment monitoring and performance drift detection.**
HHS should prioritize longitudinal studies and methodological research focused on identifying, measuring, and mitigating performance degradation over time.
- **Encourage independent and profession-led validation models.**
Third-party or profession-led testing frameworks can complement vendor-reported performance claims and strengthen trust in AI tools used in patient care.
- **Support development of standardized documentation and reporting practices.**
Clear expectations for performance reporting, update tracking, and adverse event documentation will improve transparency and accountability.

By strengthening validation standards, promoting clinically meaningful metrics, and investing in lifecycle monitoring infrastructure, HHS can ensure that AI adoption in clinical care remains evidence-based, transparent, and aligned with patient safety.

(v) How can HHS support private-sector accreditation, certification, and industry-driven testing efforts to accelerate the responsible adoption of AI in clinical care?

Private-sector organizations, including professional societies, standards bodies, and accreditation entities, play an essential role in translating emerging technologies into safe clinical practice. In domains such as medical imaging and radiation therapy, profession-led guidance, commissioning standards, and quality assurance frameworks have historically complemented federal oversight and enabled consistent implementation across diverse healthcare settings. As AI tools become more integrated into clinical workflows, similar profession-led and industry-supported assurance mechanisms will be critical to maintaining safety, consistency, and public trust. Federal policy can accelerate

responsible adoption not by duplicating these efforts, but by recognizing, aligning with, and strengthening them.

AAPM recommends the following federal actions:

- **Recognize and align with profession-led standards and guidance for AI commissioning, validation, and lifecycle oversight.**
Federal expectations should complement rather than conflict with existing clinical quality frameworks developed by professional societies and accreditation bodies.
- **Support development of accreditation and certification pathways for professionals responsible for AI oversight.**
Individuals charged with commissioning, monitoring, and governance of AI systems should have clearly defined competencies and training standards.
- **Encourage independent testing and benchmarking initiatives through public–private partnerships.**
Transparent evaluation frameworks can strengthen trust in AI tools without requiring disclosure of proprietary source code.
- **Promote coordination between HHS agencies and accreditation organizations.**
Reducing duplicative compliance requirements and harmonizing governance expectations will ease administrative burden while preserving accountability.
- **Provide grant or cooperative agreement funding to support shared testing infrastructure and governance toolkits.**
Resources that can be adopted by institutions of varying size and capacity will help ensure equitable and consistent implementation.

By supporting private-sector assurance activities and aligning federal policy with established clinical quality frameworks, HHS can strengthen accountability while enabling scalable and responsible AI adoption across the healthcare system.

(vi) Where have AI tools in clinical care met expectations, where have they fallen short, and what lessons should inform future policy and research priorities?

AI tools have demonstrated measurable benefits in several clinical domains, particularly when deployed in well-defined use cases and paired with appropriate human oversight. In medical imaging and radiation oncology, AI applications have shown value in image enhancement, automated segmentation, workflow prioritization, quality assurance support, and reduction of repetitive manual tasks. These applications can improve efficiency, increase consistency, and reduce certain types of human error when integrated thoughtfully into established clinical processes.

However, AI tools have fallen short in situations where validation has been limited, generalizability has not been adequately assessed, or workflow integration has been insufficiently planned. Performance may degrade when deployed across different patient populations, equipment platforms, or institutional settings. In some cases, overly optimistic performance claims have not translated into measurable improvements in patient outcomes or safety. These gaps underscore the importance of robust validation, ongoing monitoring, and realistic expectations regarding the capabilities and limitations of AI systems.

Economic evaluation also remains underdeveloped. While AI tools may improve workflow efficiency or reduce variability, the long-term return on investment, cost-effectiveness, and impact on patient

outcomes are not always systematically measured. Policymakers and healthcare institutions require clearer evidence regarding value to support sustainable adoption.

AAPM recommends the following federal actions:

- **Prioritize funding for real-world performance and outcomes research.**
HHS should support longitudinal studies evaluating AI tools across diverse clinical environments to assess generalizability, safety, and measurable impact on patient outcomes.
- **Expand research on economic evaluation and cost-effectiveness.**
Federal programs should encourage rigorous assessment of return on investment, workflow impact, and resource utilization associated with AI-enabled care.
- **Support comparative effectiveness research.**
Studies comparing AI-enabled workflows with optimized non-AI workflows will help clarify where AI provides meaningful clinical advantage.
- **Encourage dissemination of lessons learned from both successful and unsuccessful deployments.**
Structured mechanisms for sharing implementation experiences, including challenges and limitations, will strengthen collective learning and reduce duplication of effort.
- **Invest in implementation science focused on workflow integration and human–AI interaction.**
Understanding how AI tools influence clinician behavior, decision-making, and system-level processes is essential to realizing safe and effective adoption.

By grounding AI policy in real-world evidence, economic evaluation, and implementation science, HHS can promote innovation that delivers measurable clinical value while maintaining patient safety and public trust.

(vii) Which stakeholders and decision-makers influence the adoption of AI in clinical care, and what organizational or administrative barriers limit effective integration?

The adoption of AI in clinical care is shaped not only by regulatory and reimbursement policy, but also by internal healthcare governance structures. Decisions to procure, deploy, and monitor AI tools typically involve clinical leadership, information technology departments, compliance and privacy officers, quality and safety committees, and technical subject matter experts such as medical physicists. In safety-critical areas such as imaging and radiation oncology, AI tools must be integrated into existing quality assurance programs and clinical workflows to ensure safe and effective use.

Organizational fragmentation is a common barrier. AI deployment responsibilities are often distributed across departments without clear ownership of lifecycle oversight. Institutions may lack defined governance structures to oversee commissioning, performance monitoring, version control, and incident response. Smaller or resource-limited healthcare settings may also lack dedicated personnel with expertise in AI evaluation and quality management. Without structured governance, adoption may be inconsistent, reactive, or insufficiently documented.

Workflow integration presents additional challenges. AI tools that are technically sound may fail to deliver value if they disrupt clinical processes, create parallel workflows, or introduce ambiguity regarding clinician accountability. Effective implementation requires multidisciplinary collaboration and clearly defined escalation pathways.

AAPM recommends the following federal actions:

- **Develop model governance frameworks for healthcare institutions adopting AI.**
HHS could support creation of template oversight structures, including AI governance committees, commissioning protocols, documentation standards, and escalation pathways to guide consistent institutional practice.
- **Support workforce training and interdisciplinary collaboration.**
Federal initiatives should promote training programs that bring together clinicians, physicists, data scientists, and compliance professionals to strengthen institutional oversight capacity.
- **Fund implementation science research focused on workflow integration.**
Research evaluating how AI tools interact with clinical processes, staffing models, and decision-making pathways will help identify best practices for safe deployment.
- **Provide technical assistance resources for small and rural healthcare institutions.**
Equitable adoption requires that resource-limited settings have access to guidance and infrastructure support for safe AI integration.
- **Encourage incorporation of AI governance into existing quality and patient safety programs.**
Integrating AI oversight into established quality assurance systems will reduce duplication and reinforce accountability.

By supporting structured governance models and interdisciplinary collaboration, HHS can help healthcare organizations integrate AI in a manner that strengthens safety, accountability, and clinical effectiveness.

(viii) What data infrastructure, interoperability standards, or information-sharing approaches are needed to accelerate AI development and adoption in clinical care while protecting privacy and security?

AI development and responsible deployment in clinical care depend on access to high-quality, diverse, and well-annotated data. However, healthcare data remain fragmented across institutions, vendors, and systems, often stored in incompatible formats or subject to inconsistent metadata standards. In imaging and radiation oncology, while standards such as DICOM provide a foundation for data exchange, variability in implementation, annotation practices, and linkage to structured clinical data can limit scalability and cross-institutional validation.

Interoperability challenges affect not only model development but also lifecycle oversight. Effective AI governance requires access to performance logs, version histories, audit trails, and contextual metadata that allow institutions to evaluate how models behave across different environments. Without harmonized standards for these data elements, benchmarking and post-deployment monitoring become difficult.

Privacy and security considerations further complicate information sharing. Institutions must balance the need for diverse training and validation datasets with compliance obligations and protection of patient information. Smaller institutions may lack the infrastructure needed to participate in collaborative validation efforts, exacerbating disparities in AI development and adoption.

AAPM recommends the following federal actions:

- **Advance harmonized interoperability standards for imaging, structured clinical data, and performance metadata.**
HHS should support refinement and consistent implementation of standards that enable linkage

of imaging data, clinical context, audit logs, and versioning information necessary for AI validation and monitoring.

- **Support federated and privacy-preserving data-sharing models.**
Federal investment in federated learning frameworks and secure data enclaves can enable multi-institutional collaboration without requiring centralized data transfer.
- **Invest in shared benchmarking and validation infrastructure.**
National or regional AI evaluation hubs and shared testbeds can reduce disparities between large academic centers and smaller institutions while promoting objective comparison of AI tools.
- **Strengthen cybersecurity and secure deployment guidance for AI systems.**
Clear expectations regarding secure integration, access controls, and model integrity will protect patient data and reinforce trust in AI-enabled care.
- **Encourage integration across data commons and research repositories.**
Linking imaging data commons with electronic health record–based repositories will improve generalizability and support real-world performance evaluation across diverse populations.

By strengthening interoperability, promoting secure data-sharing models, and investing in shared infrastructure, HHS can accelerate AI innovation while protecting patient privacy and supporting equitable adoption across healthcare settings.

(ix) What challenges do patients and caregivers wish AI to address in clinical care, and what concerns should inform federal policy as AI adoption accelerates?

Patients and caregivers generally seek healthcare technologies that improve safety, access, timeliness, and consistency of care. AI has the potential to reduce diagnostic delays, decrease variability in treatment planning, enhance quality assurance, and support more efficient clinical workflows. In resource-constrained environments, AI may also help extend expertise to underserved or rural communities, potentially reducing disparities in access to high-quality care.

At the same time, patients have legitimate concerns regarding transparency, bias, accountability, and data use. Many individuals may not understand when or how AI tools are contributing to their care, raising questions about informed consent and explainability. Concerns about algorithmic bias are particularly salient, especially if training datasets do not adequately represent diverse patient populations. Patients may also worry about data privacy, security, and the potential for misuse of health information in AI development or deployment.

Trust depends not only on technological performance, but also on clear communication and sustained human oversight. Patients generally expect that clinicians remain accountable for care decisions, even when supported by AI systems. Ensuring that AI tools augment rather than replace professional judgment is critical to maintaining public confidence.

AAPM recommends the following federal actions:

- **Promote transparency regarding AI use in clinical care.**
HHS should encourage clear communication standards that inform patients when AI tools contribute to diagnosis, treatment planning, or workflow decisions in meaningful ways.
- **Support research on bias mitigation and equitable performance.**
Federal funding should prioritize evaluation of AI performance across diverse demographic groups and care settings to reduce disparities and strengthen fairness.

- **Encourage human-in-the-loop governance models.**
Policy frameworks should reinforce the expectation that clinicians retain ultimate responsibility for patient care decisions, with AI functioning as a decision-support tool.
- **Strengthen guidance on data privacy and secondary data use in AI development.**
Clarifying permissible uses and safeguards will enhance patient confidence while supporting innovation.
- **Develop patient-centered educational resources.**
HHS can support the creation of accessible materials that explain AI capabilities, limitations, and safeguards to improve public understanding.

By proactively addressing transparency, equity, and accountability, federal policy can strengthen patient trust while enabling responsible AI adoption that advances safety and quality of care.

(x) What research gaps must be addressed to accelerate the safe, effective, and equitable adoption of AI in clinical care?

While AI development in healthcare has progressed rapidly, important evidence gaps remain in real-world performance evaluation, lifecycle governance, human–AI interaction, and economic impact. Many AI tools are validated using retrospective datasets under controlled conditions, yet limited evidence exists regarding long-term performance across diverse institutions, equipment platforms, and patient populations. Without rigorous real-world evidence, policymakers and healthcare organizations may struggle to distinguish between tools that offer measurable clinical benefit and those that primarily improve workflow efficiency without affecting outcomes.

Research on lifecycle management is similarly underdeveloped. As AI systems evolve through software updates or retraining, institutions require evidence-based guidance on when revalidation is necessary and how performance drift should be detected and mitigated. Standardized methodologies for monitoring AI behavior over time remain in early stages of development.

Human–AI interaction represents another critical gap. AI tools may alter clinician decision-making patterns, workload distribution, and escalation practices in ways that are not fully understood. Evaluating how clinicians interpret AI outputs, manage uncertainty, and respond to algorithmic recommendations is essential to ensuring safety and accountability.

Economic evaluation is also needed. Healthcare systems require clearer evidence regarding cost-effectiveness, return on investment, and impact on resource utilization. Without systematic economic analysis, adoption decisions may be driven by short-term efficiency gains rather than long-term patient benefit.

AAPM recommends the following federal actions:

- **Prioritize funding for real-world performance and generalizability studies.**
HHS should support longitudinal, multi-institutional research evaluating AI tools across diverse patient populations and care environments.
- **Invest in lifecycle monitoring and performance drift methodologies.**
Research focused on update management, retraining triggers, and standardized revalidation protocols will strengthen long-term oversight.
- **Expand support for human–AI interaction and implementation science research.**
Understanding how AI tools influence clinician behavior and workflow integration is essential to maximizing safety and effectiveness.

- **Support rigorous economic evaluation studies.**
Federal programs should fund cost-effectiveness analyses and return-on-investment studies to inform sustainable adoption decisions.
- **Encourage development of shared benchmarks and reporting standards.**
Common evaluation frameworks will promote transparency and comparability across AI tools and healthcare settings.

By addressing these research gaps through coordinated federal investment, HHS can build the evidence base necessary to support safe, accountable, and value-driven integration of AI into clinical care.

For additional information or questions, please contact Lauren DePutter, AAPM's Director of Government Affairs and External Relations, at lauren@aapm.org.